

Two axes, not one: venous outflow, the long flaccid state, and the orthostatic assumption in hard flaccid syndrome

Jesterhfs¹ and Claude Opus 5²

¹HFS Patient • ²Large language model, Anthropic PBC

Fourteenth and last in a series examining the conjectures of *Fourteen conjectures on the mechanism*.

Abstract

Background. The final conjecture in this series proposes that a subset of patients has a genuine venous outflow component accounting for engorged veins, discoloration, and the distended, fluid-filled quality of the reported “long flaccid” state. Its author rates it low to moderate and identifies its weakness as undetermined causal direction.

The observation that earns the conjecture its place. Contraction shortens. A penis that is *longer* than it should be cannot be explained by excess smooth muscle contraction, and this is the only symptom in the entire set that is mechanically incompatible with the framework’s founding premise. That deserves more weight than it has been given.

But it needs splitting. Long-and-soft requires no venous mechanism at all: it is simply the low-tone end of the ordinary tone axis, since normal flaccid tone shortens the organ. Only *long-and-firm* — distended and tense — requires trapped volume. Separating these two descriptions turns one vague symptom into a discriminating one, and we set out the tone–volume plane on which hard flaccid, long flaccid and normal flaccidity are distinct points rather than a spectrum.

The base-rate problem is worse than stated, and quantifiable. Asymptomatic compression of the left renal vein of 50% or greater is reported in 51–72% of computed tomography angiograms. The vascular field has explicitly moved away from the terms pelvic congestion, May–Turner and nutcracker *syndrome* on the grounds that such nomenclature conflates anatomical findings with pathology. A “frequently positive” finding with a background prevalence between one half and three quarters carries no information without matched controls.

The relevant test has already been performed and the conjecture does not cite it. Penile veno-occlusive dysfunction is diagnosed on duplex Doppler by a persistently elevated end-diastolic velocity above roughly 5 cm/s with preserved arterial inflow. In the largest reported hard flaccid syndrome series, all 88 patients had normal peak systolic velocity and **2 of 88 (2.3%) had abnormal end-diastolic velocity** — a near-negative result for penile venous incompetence, drawn from the same systematic review the conjecture cites for its supporting observations.

A contribution beyond this conjecture. The conjecture correctly notes that gravitational venous pooling predicts orthostatic worsening just as sympathetic activation does. That undermines an assumption used throughout the whole framework. We propose a resolution: a supine cold pressor test raises sympathetic outflow substantially without any change in posture or pelvic venous filling. If symptoms worsen supine under sympathoexcitation, the orthostatic observation is sympathetic; if they do not, it is hydrostatic. The test is free, non-invasive, and settles a premise fourteen conjectures depend on.

Keywords: hard flaccid syndrome; long flaccid; pelvic venous disorders; veno-occlusive dysfunction; end-diastolic velocity; orthostatic worsening; cold pressor test.

Reader advisory

This is hypothesis generation, not a report of new data. No component of the mechanism proposed here has been demonstrated in a person with hard flaccid syndrome. This paper discusses pelvic venous imaging and embolisation only as research and diagnostic questions; **nothing here supports seeking venous embolisation, stenting or any vascular intervention for this condition**, and such procedures carry real risk and have not been shown to help it. Nothing here should influence clinical management.

1. Introduction

This is the last of fourteen conjectures and the only one that is primarily vascular. It proposes that a subset of patients has a genuine venous outflow component, accounting for engorged and prominent veins, penile skin discoloration, and

the distended, fluid-filled quality of what patients call the “long flaccid” state [4].

The conjecture is rated low to moderate by its author, who identifies its central weakness accurately: the causal direction is undetermined, and the reported frequent positive findings for pelvic venous congestion are compatible with congestion following sympathetic change, with congestion contributing independently, and with neither being causal [4].

We agree with that assessment and go further in two directions. The base-rate problem is quantifiable and is worse than the conjecture states. And the specific test that bears most directly on the hypothesis has already been performed in the largest reported series, with a result the conjecture does not mention.

But the conjecture also contains an observation that we think is underrated, and which no other conjecture in the set can accommodate. It is worth stating on its own.

2. Methods

This is a hypothesis and theory article. No new human or animal data were generated; no patients were involved; ethical approval was not required.

Literature was identified by targeted searching of MEDLINE/PubMed and the open web between May and August 2026, using combinations of the terms *pelvic venous disorders*, *pelvic congestion syndrome*, *nutcracker phenomenon prevalence*, *penile duplex Doppler end-diastolic velocity*, *veno-occlusive dysfunction*, and *hard flaccid syndrome*, with backward citation chasing. The search was purposive and non-systematic. We searched specifically for base-rate data on incidental pelvic venous findings and for the diagnostic criteria for penile veno-occlusive dysfunction, and Sections 4 and 5 report what that returned.

Claims are graded as in Table 1. Claims not verified against a retrievable source are marked [VERIFY]; quantities that do not exist in the literature are marked [DATA NEEDED].

Table 1. Evidence grading scheme. The grade refers to the strength of evidence for the *stated proposition*, not to its relevance to hard flaccid syndrome. No proposition here carries grade A evidence *in HFS*, because none exists.

Grade	Definition
[A]	Replicated human evidence, clinical guideline, consensus classification, or uncontested physiology.
[B]	Single human study, congress abstract, or human data with substantial methodological limitation.
[C]	Experimental animal or in vitro evidence.
[D]	Mechanistic or physical inference: deduced from A–C evidence but not itself directly observed.
[E]	Unverified anecdotal or community-sourced report; not peer reviewed, denominators unknown.

3. The observation the conjecture gets right

3.1 Contraction shortens

The founding premise of this entire framework is that the hard flaccid state is sustained contraction of cavernosal smooth muscle [4]. Contraction of the trabecular smooth muscle expels blood from the sinusoids and reduces corporal volume; the resulting penis is smaller and firmer [A] [7, 8].

A penis reported as *longer* than it should be is therefore not explicable by excess contraction. **This is the only reported symptom in the whole set that is mechanically incompatible with the framework’s central premise**, and the conjecture is right to flag it. A theory that can absorb every observation is weaker than one that identifies an observation it cannot absorb [D].

3.2 But two different things are being described

The conjecture describes the long flaccid state as “a penis more extended than it should be, feeling either firm or like a balloon filled with water” [4]. Those are not one phenomenon, and separating them does real work.

- **Long and soft** requires no venous mechanism. Normal flaccidity is itself an actively contracted state [A] [7]; *reducing* that tone lengthens and softens the organ. A long, soft, pendulous penis is simply the low-tone end of the ordinary tone axis, and needs no trapped blood to explain it [D].
- **Long and firm** — distended, tense, water-filled — does require volume. Neither high tone nor low tone produces a penis that is simultaneously enlarged and tense. That combination requires blood retained against outflow resistance [D].

This refinement matters because it changes what should be recorded. **Asking patients whether the long state is**

soft or tense converts a vague symptom into one that discriminates between a tone mechanism and a volume mechanism, at no cost. So far as we can establish, no study has made the distinction [DATA NEEDED].

3.3 Tone and volume are separate axes

The framework has largely treated the penile state as one-dimensional: more tone or less. The long flaccid observation forces a second dimension. Figure 1 sets out the tone–volume plane, on which normal flaccidity, the hard flaccid state, the two long flaccid variants and erection are distinct points rather than positions on a single line.

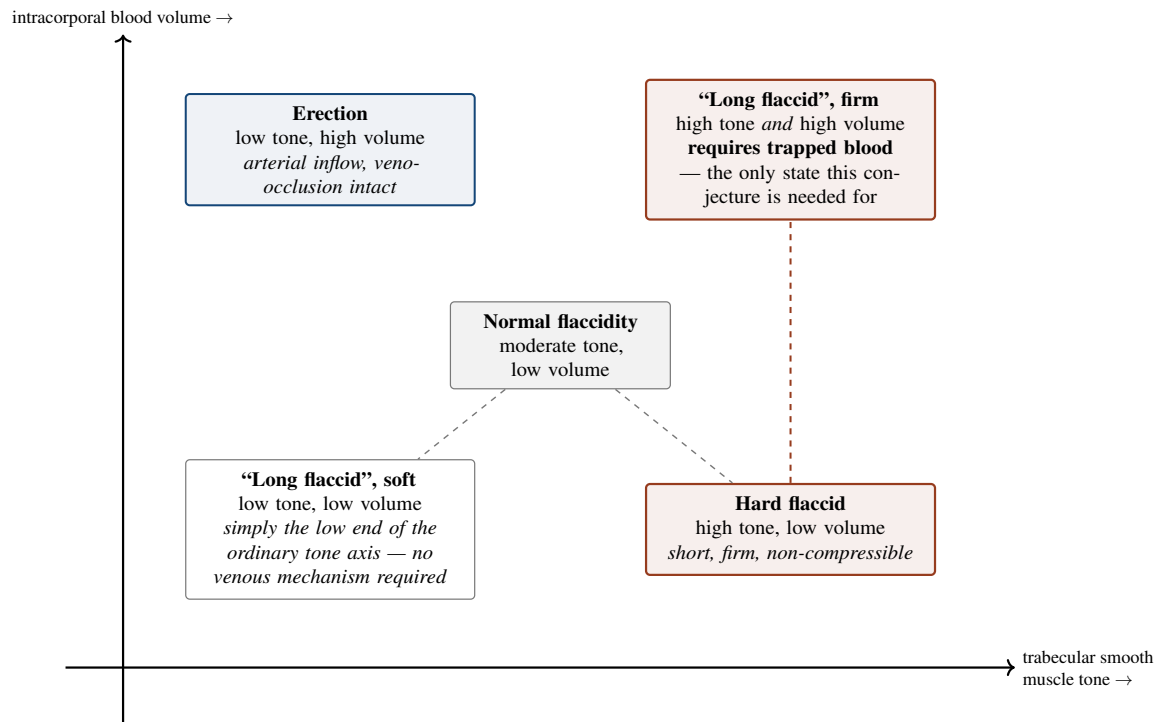


Figure 1. The tone–volume plane. Treating the penile state as one-dimensional — more tone or less — cannot accommodate a penis that is simultaneously longer and firmer than normal. On two axes the states separate cleanly. The horizontal dashed line from normal flaccidity to the hard flaccid state is what thirteen of the fourteen conjectures address. The long-and-soft state lies on the same axis in the opposite direction and requires no new mechanism. Only the long-and-firm state (upper right) requires retained volume against outflow resistance, and it is the only observation for which this conjecture is necessary. Recording which of the two long states a patient describes therefore discriminates between a tone mechanism and a volume mechanism at no cost. Diagram is conceptual; the axes are not quantified and no measurements of either exist in this condition.

4. The base-rate problem, quantified

The conjecture notes that “reported frequent positive findings for pelvic venous congestion” occur in asymptomatic people and prove nothing without matched controls [4]. That is correct, and the magnitude can be put on it.

Asymptomatic compression of the left renal vein of 50% or greater — the nutcracker phenomenon — is seen in 51% to 72% of computed tomography angiograms [A] [VERIFY] [10]. A finding present in between half and three quarters of people who do not have the symptom cannot, by its presence, explain the symptom in anyone.

The vascular field has drawn the corresponding conclusion at the level of nomenclature. An international multidisciplinary working group convened to develop a classification for pelvic venous disorders stated that misleading historical nomenclature — “such as the May–Thurner, pelvic congestion, and nutcracker syndromes” — often fails to recognise the interrelationship of pelvic symptoms and their underlying pathophysiology, and produced the Symptoms–Varices–Pathophysiology classification in response [A] [VERIFY] [10]. Others have noted that the lack of validated definitions and established imaging criteria, and ambiguous cause-and-effect relationships, have prevented acceptance of the term pelvic congestion syndrome by some medical societies [B] [VERIFY] [11].

The lesson generalises well beyond this conjecture. A field that spent decades naming syndromes after anatomical findings has had to reverse course because the findings turned out to be common in people without the syndrome. Hard flaccid syndrome is at an earlier point on the same road, and the specific error — treating a prevalent anatomical variant as an explanation — is one it is currently positioned to repeat [D].

5. The relevant test has already been done

Penile veno-occlusive function is assessed routinely. On duplex Doppler after intracavernosal vasoactive injection, veno-occlusive incompetence is indicated by a persistently elevated end-diastolic velocity — thresholds of approximately 4.5–5 cm/s are used — in the presence of preserved arterial inflow, with a resistance index below about 0.75–0.8 as a supporting criterion [A] [VERIFY] [13, 14, 15]. The diagnosis can only be made when arterial function is normal [A] [VERIFY] [14].

In the largest reported hard flaccid syndrome series — 88 men undergoing penile colour Doppler ultrasound — **all had normal peak systolic velocity, and 2 of 88 (2.3%) had abnormal end-diastolic velocity** [B] [3, 1].

That is the test. Arterial inflow was preserved in every patient, which is the precondition for interpreting the venous measure, and the venous measure was abnormal in one patient in forty-three. **This is a near-negative result for penile veno-occlusive dysfunction in hard flaccid syndrome, and it comes from the same systematic review the conjecture cites for its supporting observations** [1].

Three qualifications, stated because they are real and not because they rescue the hypothesis:

1. **The measurement is made in an artificially erect penis.** Duplex Doppler assesses veno-occlusive competence during a pharmacologically induced erection. It does not measure venous outflow in the flaccid or partially tumescent state, which is where the long flaccid phenomenon is reported [D]. This is a genuine limitation and is the strongest available defence of the conjecture.
2. **The series is a congress abstract** with no control group and no reported symptom duration or subtype [3]. Whether any of the 88 described a long flaccid presentation is unknown [DATA NEEDED].
3. **Penile veno-occlusive function is not pelvic venous function.** A normal corporal veno-occlusive mechanism does not exclude proximal pelvic venous pathology, and the conjecture concerns both [D].

Even allowing all three, the finding is the most relevant human datum available and it points away from the hypothesis. It should be cited by anyone advancing this conjecture.

6. The orthostatic assumption, and how to settle it

The conjecture makes an observation whose implications reach beyond itself, and which we think is the most valuable thing in it after the shortening argument.

6.1 The problem

The framework's third declared assumption is that worsening on standing reflects the physiological increase in sympathetic outflow accompanying upright posture, supported by the established rise in muscle sympathetic nerve activity under orthostatic stress through baroreflex resetting [A] [9, 4]. The supporting symptom data are strong: worsening on standing in 64.5% and improvement lying down in 73.0% of 143 surveyed patients [B] [2].

The conjecture points out that gravitational venous pooling occurs simultaneously and predicts the same symptom [4]. **Standing raises sympathetic outflow and increases dependent venous filling at the same time, by different mechanisms, in every person.** The orthostatic observation is therefore consistent with the sympathetic account without being diagnostic of it — and by the same token it is consistent with a venous account without being diagnostic of that either [D].

This is a conjecture undermining a premise on which the other thirteen rest. It deserves more than the single paragraph it receives.

6.2 A test that separates them

The two mechanisms can be dissociated because sympathetic outflow can be raised without changing posture.

Prediction P1 (the supine cold pressor test). A standardised cold pressor test performed *supine* raises sympathetic outflow substantially while leaving posture, hydrostatic gradient and pelvic venous filling unchanged. If the hard flaccid state worsens under supine sympathoexcitation, the orthostatic observation is sympathetic. If it does not worsen supine but does on standing, the orthostatic observation is hydrostatic.

Prediction P2 (compression control). Symptoms on standing are attenuated by graduated lower-limb and abdominal compression, which counteracts dependent pooling with comparatively little effect on sympathetic outflow. Attenuation favours the venous account; no attenuation favours the sympathetic one.

These are cheap, non-invasive, repeatable within a single session, and require no drug and no procedure. Both depend on the endpoint problem that has limited every paper in this series — no validated objective measure of resting cavernosal tone exists [DATA NEEDED] [5, 6] — but both would be informative even with a pre-specified blinded

patient-rated scale, because the comparison is within-subject and the two conditions can be counterbalanced.

We regard this as the single most valuable study proposed in this paper, because it does not test this conjecture. It tests an assumption that fourteen conjectures share.

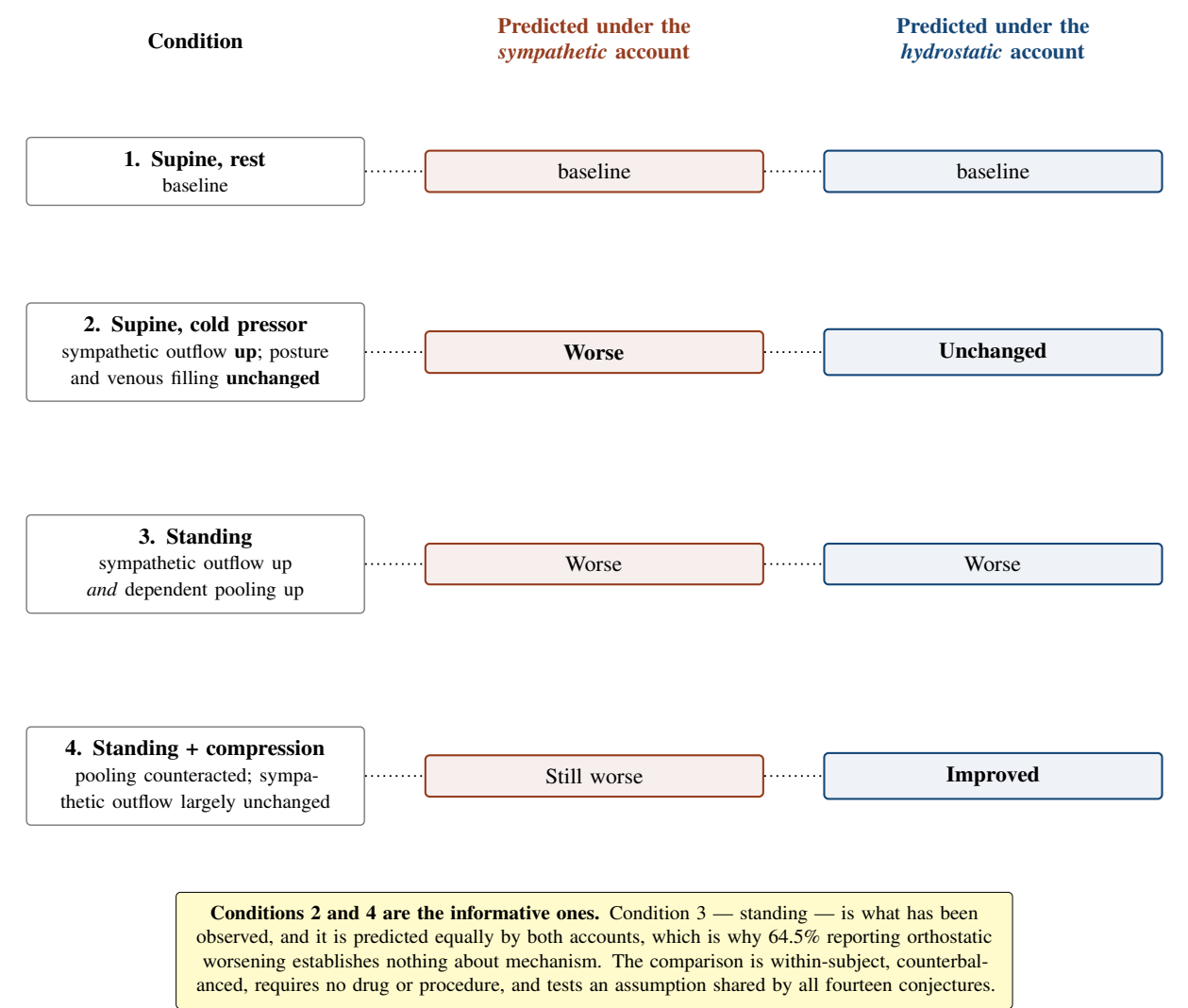


Figure 2. Separating sympathetic from hydrostatic explanations of orthostatic worsening. Standing raises sympathetic outflow and dependent venous filling simultaneously, so the observed orthostatic worsening is uninformative about mechanism. A supine cold pressor test raises sympathetic outflow without changing posture; graduated compression counteracts pooling without much changing sympathetic outflow. The two manipulations dissociate the accounts. Both require a tone endpoint, which does not yet exist in validated form, but both are informative even with a pre-specified blinded patient-rated measure because the design is within-subject.

7. The hypothesis

Formal statement, with the split applied

H14 (venous outflow contribution). In a subset of patients with hard flaccid syndrome, elevated venous outflow resistance or increased venous filling contributes to the penile findings, accounting for prominent veins, discoloration, and a distended, tense penile state.

H14-corporal. The lesion is corporal veno-occlusive: blood is retained within the corpora against outflow resistance. Prediction: elevated end-diastolic velocity on duplex Doppler. **Tested in 88 patients; abnormal in 2 (2.3%)** [3].

H14-pelvic. The lesion is proximal — pelvic venous obstruction or reflux raising outflow resistance from the penis. Prediction: pelvic venous abnormality exceeding the rate in matched asymptomatic controls, and correlating with the tense-and-long presentation rather than with the hard flaccid state.

H14-consequence. Venous findings are downstream of chronic sympathetic change in pelvic venous tone. Prediction: findings track symptom duration rather than presentation subtype, and do not precede the condition.

H14-null. Venous findings in this population occur at the background population rate; the long-and-soft state is low tone and the long-and-firm description does not survive measurement.

Auxiliary assumptions:

- **A1 (the state is real).** That a penis both longer and firmer than normal occurs and is reproducible. Currently grade E self-report; never measured [DATA NEEDED].
- **A2 (subset structure).** That patients partition into tone-dominant and congestion-dominant groups rather than the descriptions reflecting variable wording for one state. Untested; the conjecture's own suggestion that individuals alternate between them [4] would, if true, argue against a stable subset and for a within-person axis.
- **A3 (venous findings are not incidental).** Given the 51–72% background prevalence of the commonest such finding [10], this assumption cannot be evaluated at all without matched controls.

8. Predictions and tests

8.1 The free test: record which long state

Prediction P3. Patients describing the long flaccid state will separate into a soft group and a tense group, and pelvic or corporal venous findings will be concentrated in the tense group.

This requires only that the question be asked. No existing study distinguishes the two [DATA NEEDED], and the distinction determines whether a venous mechanism is needed at all (Section 3.2, Figure 1).

8.2 Imaging, only with controls

Prediction P4. Pelvic magnetic resonance venography in men with HFS, read blind against age-matched asymptomatic controls imaged identically, will show a higher rate of pelvic venous reflux or obstruction than controls.

The control group is not optional and is the whole study. Given a background prevalence of 51–72% for asymptomatic left renal vein compression [10], an uncontrolled series reporting frequent positive findings would be uninterpretable however large. **We would not fund an imaging study of this question without a matched control arm, and we would regard an uncontrolled one as actively misleading.**

Prediction P5. Penile duplex Doppler performed supine and standing in the same patients will show a state-dependent difference in venous parameters. This addresses the principal limitation of the existing data (Section 5) — that duplex is performed in an artificially erect penis — and is the most direct available test of the conjecture as stated. Its feasibility is uncertain: standing duplex is not a standard protocol and the reproducibility of venous measures in that position is unestablished [DATA NEEDED].

8.3 The test that matters most, and is not about this conjecture

Prediction P1, the supine cold pressor test (Section 6). It is the cheapest study proposed anywhere in this series and it interrogates the premise all fourteen conjectures share.

9. Alternative explanations

1. **Low tone, not high volume.** The long-and-soft state requires no venous mechanism (Section 3.2). If most reports of “long flaccid” describe a soft penis, the conjecture explains nothing that the tone axis does not.

Table 2. Predictions of H14 compared with the principal alternatives.

Observation	H14 (venous)	Tone mechanisms (I–VIII)	Structural (IX–XI)	Discriminating value
Penis longer <i>and</i> firmer than normal	Predicted	Refuted (contraction shortens)	Not predicted	High — the conjecture’s reason to exist
Penis longer and <i>softer</i>	Not needed	Predicted (low tone)	Not predicted	High, against H14
Elevated end-diastolic velocity on duplex	Predicted	Not predicted	Possible	High — and 2/88 observed
Pelvic venous findings exceeding matched controls	Predicted	Not predicted	Not predicted	High, but never tested with controls
Pelvic venous findings at the population rate	Refutes	—	—	Decisive, and cheaply obtainable
Worsening on standing	Predicted	Predicted	Not specified	None
Worsening on supine cold pressor	Not predicted	Predicted	Not predicted	High
Standing symptoms attenuated by compression	Predicted	Not predicted	Not predicted	High

2. **Incidental anatomical variants.** Given a 51–72% background prevalence for the commonest pelvic venous finding [10], positive findings in a patient series are the expected result of imaging anyone.
3. **Ascertainment.** Men with a distressing genital condition who seek investigation will be imaged more often than the general population, and findings present at high background prevalence will therefore be discovered and attributed [D]. This is a mechanism for generating apparent association from nothing at all, and it operates strongly in conditions with an engaged, well-informed patient community.
4. **Reverse causation.** Chronic sympathetic change in pelvic venous tone could produce venous findings downstream of the condition, as the conjecture itself notes [4]. No cross-sectional design distinguishes this.
5. **Prominent veins have simpler explanations.** Visible superficial dorsal and subcutaneous veins vary with body composition, skin thickness, hydration, temperature and posture, and are conspicuous to a person examining himself closely. Discoloration likewise. Neither observation requires a venous outflow lesion [D].
6. **The premise may be false.** As in every paper in this series: cavernosal tone has never been objectively shown to be elevated in HFS, and no measurement of penile dimensions across states has ever been made in an affected person.

10. Implications

No clinical implications are drawn, and we state explicitly that nothing here supports pursuing venous embolisation, iliac stenting or any vascular procedure for this condition. Those interventions are contested even in the populations for which they were developed, and applying them on the basis of a finding present in most asymptomatic people would be a predictable route to harm.

Three research implications follow.

First, ask which long state. Soft or tense. The distinction determines whether a venous mechanism is required, and it costs one question.

Second, never image without controls. The base rate makes uncontrolled imaging of this question worse than useless, because it will reliably generate positive findings that mean nothing.

Third, do the cold pressor study. It settles whether orthostatic worsening — the observation cited most often across this whole framework — is sympathetic or hydrostatic. Nothing else proposed in fourteen papers is as cheap or bears on as much.

11. Limitations

1. **No evidence in HFS.** Pelvic venous imaging with controls has never been performed in this population, and penile dimensions have never been measured across states.
2. **The relevant existing datum points away from the hypothesis:** 2 of 88 with abnormal end-diastolic velocity [3].
3. **The base rate defeats uncontrolled evidence entirely,** and all evidence currently offered for this conjecture is uncontrolled.
4. **The motivating symptom is grade E.** “Long flaccid” appears in community description and is not, so far as we can establish, characterised in any peer-reviewed source [DATA NEEDED].
5. **The subset structure is unevidenced,** and the conjecture’s own suggestion that patients alternate between states argues against stable subsets.

6. **Duplex Doppler measures the wrong state** — an artificially erect penis — which is the strongest available defence of the conjecture and also means the existing near-negative result is not decisive.
7. **The proposed orthostatic tests still lack an endpoint**, as does everything in this series.
8. **Several citations are flagged unverified**, including the base-rate figure that carries most of the critical argument; it should be confirmed against the primary source before being quoted.
9. **We may be over-reading the shortening argument**. That contraction shortens is elementary physics of the tissue, but penile length in the flaccid state also depends on skin, dartos, fascial and suspensory factors that we have not considered [D].
10. **Confirmation risk, inverted**. This paper is largely critical of the conjecture it examines, and a critical stance carries its own selection pressure. We have tried to counter it by identifying the observation the conjecture gets right and by giving the strongest defence of the negative Doppler finding, but readers should discount accordingly.

12. Refutation criteria

- **Refuted**. Pelvic venous findings in men with HFS occur at the same rate as in age-matched asymptomatic controls imaged and read identically.
- **Substantially refuted**. The reported long flaccid state proves on measurement to be uniformly soft, which requires only reduced tone.
- **Substantially refuted**. Penile duplex Doppler performed supine and standing shows no state-dependent venous abnormality.
- **Weakened further**. Symptoms on standing are unaffected by graduated compression.
- **Demoted to H14-consequence**. Venous findings track symptom duration rather than presentation subtype.
- **Rendered moot**. Direct measurement shows penile dimensions and resting tone in symptomatic patients do not differ from controls.

13. Conclusion

This conjecture is right about one thing that matters and is otherwise in a weaker position than its own author acknowledges.

What it gets right is elementary and important: contraction shortens. A penis reported as longer than it should be cannot be produced by excess smooth muscle contraction, and that makes it the only symptom in the whole set mechanically incompatible with the framework's founding premise. But the observation needs splitting. Long-and-soft is simply the low-tone end of the ordinary axis and requires nothing new, because normal flaccidity is itself a contracted state and reducing tone lengthens the organ. Only long-and-*firm* requires blood held against outflow resistance. Asking patients which they mean converts a vague symptom into a discriminating one, and nobody has asked.

What it is wrong about is the weight the venous findings can bear. Asymptomatic left renal vein compression of 50% or more appears in 51 to 72 per cent of computed tomography angiograms, and the vascular field has formally abandoned the syndrome names built on such findings because they conflated anatomy with pathology. Against that background, "frequently positive" is not evidence. And the test that bears most directly on the hypothesis has already been done: in 88 men with hard flaccid syndrome, arterial inflow was normal in all — the precondition for reading the venous measure — and end-diastolic velocity was abnormal in two. That result appears in the same systematic review the conjecture draws on, and it is not mentioned.

The most valuable thing in this conjecture is not about this conjecture. It observes that gravitational venous pooling predicts orthostatic worsening just as sympathetic activation does, and therefore that the most frequently cited observation in the entire framework — 64.5 per cent worse standing, 73 per cent better lying down — is consistent with the sympathetic account without being diagnostic of it. That is a premise thirteen other conjectures rest on. It can be settled by raising sympathetic outflow without moving the patient: a cold pressor test performed supine. If the state worsens, the orthostatic observation is sympathetic. If it does not, fourteen papers have been building on a hydrostatic artefact.

That study needs no drug, no procedure, no imaging and no new instrument. It is the cheapest thing proposed in this series and it bears on the most. It is a fitting place for the series to end: not with a new mechanism, but with the observation that the field's most quoted clinical fact has never been tested against the most obvious alternative explanation for it.

Declarations

Authorship and the use of artificial intelligence. The second listed author is a large language model. It is listed at the corresponding author's request and in the interest of transparency, but this contravenes the recommendations of the International Committee of Medical Journal Editors, which hold that artificial intelligence tools cannot meet authorship criteria because they cannot take responsibility for the accuracy and integrity of the work. **Before submission to any peer-reviewed venue this attribution should be converted into an AI-use disclosure, with the human author accepting sole responsibility for the content.** The 51–72% base-rate figure carries most of the critical argument in this paper and must be confirmed against the primary source before it is quoted in print.

Ethics approval. Not applicable. No human participants, animals or identifiable data were involved.

Funding. None.

Competing interests. No competing financial interests. The first author maintains a public website on hard flaccid syndrome and authored the source article [4]; this is a non-financial interest in the hypothesis being taken seriously, and readers should weigh the argument accordingly.

Data availability. No datasets were generated or analysed.

Clinical disclaimer. This article is hypothesis generation. It does not constitute medical advice and does not establish a clinician–patient relationship. **Nothing in it supports pursuing pelvic venous embolisation, iliac vein stenting, penile venous ligation or any other vascular intervention for hard flaccid syndrome.** These are invasive procedures with real risks whose benefit is contested even in the conditions for which they were developed, and no evidence supports their use here. Penile duplex ultrasound with pharmacologically induced erection is an invasive procedure appropriate only under qualified medical supervision.

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